

Head Injury (Canadian CT Head Rule) - Female (Adult)

Algorithm 1048 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only adult minor-head-injury pathway describing the Canadian CT Head Rule and routing for in-person assessment; it must not decide that CT or clinical review is unnecessary by telephone.
- Adult patient.
- Female.

Clinical background

- The original Canadian CT Head Rule applies after blunt head trauma to patients aged 16 years or older with GCS 13-15 and witnessed loss of consciousness, definite amnesia or witnessed disorientation. It does not apply to minimal head injury without those entry features, age under 16, penetrating/depressed skull injury, acute focal neurological deficit, unstable major trauma, bleeding disorder/oral anticoagulant use, or seizure after injury. This adult UAT pathway uses age 18 and older. GCS, skull-fracture signs and neurological findings require in-person examination; exclusion from the rule is not reassurance.

Pain severity

- Mild (1-3): Does not interfere with normal activity, movement, concentration, or sleep.
- Moderate (4-7): Interferes with normal activity, movement, concentration, or sleep.
- Severe (8-10): Prevents normal activity or movement, or is described as excruciating.

Initial assessment

1. [YES_NO] Did the patient lose consciousness, and for how long?
2. [DURATION] When did the injury happen?
3. [OPEN_TEXT] How did the injury happen, including height, stairs, vehicle involvement, penetrating injury or other major trauma?
4. [YES_NO] Is the patient under 18, pregnant, taking an anticoagulant or antiplatelet medicine, known to have a bleeding disorder, or did a seizure occur after injury?
5. [PAIN_SEVERITY] What is the current pain severity from 0 to 10, and how is it affecting normal activity or movement?
6. [PREGNANCY] Is the patient pregnant or possibly pregnant? If yes, record gestation where known.

Triage and redirect questions

1. Redirect → Headache

Is the main problem a headache without a recent blow, fall, collision, or other head injury?

The Canadian CT Head Rule applies to a defined minor-head-injury population and must not be applied to a non-traumatic headache.

1. Disposition level 100

Is the patient difficult to wake, increasingly confused, having a seizure, weak or numb on one side, unable to speak/walk normally, repeatedly vomiting, deteriorating, or showing suspected open/depressed or basal skull fracture, penetrating injury, uncontrolled bleeding, or unstable major trauma?

These are emergency danger signs or populations needing immediate stabilization, not a telephone Canadian CT Head Rule calculation. Call 999 regardless of a reported score.

1. Disposition level 70

For the in-person clinician: is GCS below 15 at two hours, is an open/depressed or basal skull fracture suspected, has vomiting occurred two or more times, or is age 65 years or older?

Published Canadian CT Head Rule high-risk criteria (Stiell et al., 2001) - associated with need for neurosurgical intervention.

2. Disposition level 70

For the in-person clinician: is there retrograde amnesia longer than 30 minutes, or a dangerous mechanism (pedestrian struck by motor vehicle, occupant ejected from motor vehicle, or fall from at least 3 feet / 5 stairs)?

Published Canadian CT Head Rule medium-risk criteria - associated with clinically important brain injury on CT.

1. Disposition level 70

Even if no criterion is reported, has the adult not had an in-person GCS, neurological and skull assessment and confirmation that the rule's entry criteria and exclusions are satisfied?

Telephone proxies cannot establish a validated rule-negative result. Patients outside the rule, including those on anticoagulants or antiplatelets, need clinician-directed assessment rather than remote scoring. Children require a separate pediatric head-injury pathway.

Advice

Emergency head injury precautions

- Call Qatar emergency services on 999. Do not allow self-driving. Keep the airway clear and follow the 999 call-taker; if unconscious but breathing and no spinal concern prevents it, use the recovery position. Do not give food, drink or unprescribed medicine while emergency assessment is being arranged. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening confusion; new weakness; repeated vomiting.

Rule criterion or exclusion - in-person assessment

- Arrange prompt in-person assessment at a Qatar service able to perform GCS, neurological/skull examination and CT when clinically indicated. A responsible adult should stay with the patient. Do not stop prescribed anticoagulant or antiplatelet medicine unless the treating clinician directs it. Pregnancy must be disclosed but must not delay emergency care; CT risk-benefit belongs to the treating team.
- Call Back If: increasing confusion or drowsiness; repeated vomiting; worsening headache.

Telephone assessment cannot make the rule negative

- Arrange in-person assessment; do not use this telephone pathway to decide that CT or observation is unnecessary. Avoid alcohol, driving, sport and sedating non-prescribed medicines while awaiting review. The exact non-emergency Qatar destination and observation/discharge instructions are GOVERNANCE_REQUIRED.
- Call Back If: worsening headache; repeated vomiting develops; increasing confusion, drowsiness, or new weakness.

First aid

- Call Qatar emergency services on 999. Do not allow self-driving. Keep the airway clear and follow the 999 call-taker; if unconscious but breathing and no spinal concern prevents it, use the recovery position. Do not give food, drink or unprescribed medicine while emergency assessment is being arranged. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Stiell IG, Wells GA, Vandemheen K, et al. The Canadian CT Head Rule for patients with minor head injury. Lancet. 2001;357(9266):1391-1396.
- National Institute for Health and Care Excellence. Head injury: assessment and early management (NG232). Published 2023; updated 2025.

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source clinical decision-rule content derived from the cited peer-reviewed publications; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Telephone adaptations, local disposition mappings, and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 06932fa9b054c860fc80b011bd5075fce2655d4f13f6edba9e32350b39c240d9 | Status: BLOCKED_BY_RESEARCH_GAP